INFORMATION TO KEEP WITH YOU

My complete treatment handbook

FOLFOX-4

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
14 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00210
Version 9b

Day 1: oxaliplatin and folinic acid, then fluorouracil bolus and a 22-hour infusion. Day 2: further fluorouracil bolus and a second 22-hour infusion; completion around Day 3. A repeating 14-day cycle. For adjuvant FOLFOX-4, NCCP describes 12 cycles; metastatic treatment continues while suitable.

Treatment stages

01 **A 14-day reference cycle**
Clinic → home → review

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

FOLFOX-4 · National Cancer Control Programme (NCCP) 00210

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. New breathing difficulty, chest pain, collapse, severe diarrhoea or inability to drink require prompt assessment. Call for line pain, swelling, leakage or unexpected pump progress.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

How your medicines work

FOLFOX-4 may be given after surgery for stage II or III colon cancer, or for colorectal cancer that has spread. Your team explains the aim, alternatives and how benefit will be reviewed.

PLATINUM CHEMOTHERAPY

Oxaliplatin

Eloxatin



Links deoxyribonucleic acid (the cell's genetic material) (DNA) strands

Oxaliplatin makes links within DNA. These interfere with copying and repair, which can stop a cancer cell dividing. Healthy tissues can also be affected.

Platinum enters cell → DNA links form → Copying is disrupted

What to watch for

Cold can trigger tingling or throat discomfort. Tell the team about persistent numbness, difficulty with buttons or walking, or infusion symptoms. New breathing difficulty always needs assessment.

CHEMOTHERAPY MODULATOR

Folinic acid

Calcium leucovorin



Strengthens fluorouracil action

Folinic acid helps fluorouracil remain bound to an enzyme needed to make DNA. In this regimen it increases fluorouracil activity and can also increase its unwanted effects. It is not a vitamin supplement to start yourself.

Folinic acid → Stronger enzyme binding → Fluorouracil effect increases

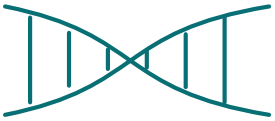
What to watch for

The combined treatment can increase mouth soreness and diarrhoea. Report these promptly. Rare allergic reactions can occur.

CHEMOTHERAPY

Fluorouracil

5-FU



Disrupts cell building blocks

Fluorouracil interferes with the production and use of DNA and RNA, the instructions cells need to grow. The infusion extends exposure over approximately two days.

Drug enters cell → DNA / RNA disrupted → Growth slows

What to watch for

Diarrhoea, mouth sores and skin soreness may become serious. Chest pain during the pump infusion needs emergency assessment. Unexpected severe early toxicity can occur even after dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) testing.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim

The setting changes what benefit means.

A plan that can change

Ask whether the aim is to reduce recurrence risk, control cancer, relieve symptoms or support a local treatment. Your preferences and priorities matter.

Understanding the evidence

Ask which evidence matches your cancer, treatment aim and medicines.

The MOSAIC trial found a long-term survival benefit from adding oxaliplatin for people with stage III colon cancer after surgery. The same survival benefit was not established in its stage II group. Metastatic treatment has a different aim and evidence discussion.

SACTCHECK · CHAPTER 02

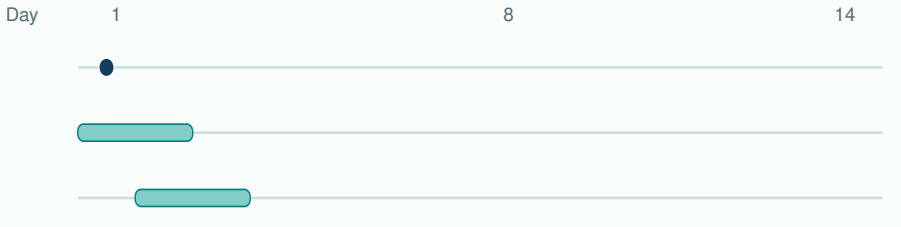
Your schedule, tests and appointments

FOLFOX-4 · NCCP 00210

Treatment, recovery and review

FOLFOX-4

14-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

CLINIC → HOME → REVIEW

A 14-day reference cycle

D1 D2 D3 D14

Day 1: oxaliplatin and folinic acid, then fluorouracil bolus and a 22-hour infusion. Day 2: further fluorouracil bolus and a second 22-hour infusion; completion around Day 3. A repeating 14-day cycle. For adjuvant FOLFOX-4, NCCP describes 12 cycles; metastatic treatment continues while suitable.

This is a general explanation, not an appointment calendar. Delays or changes are decided by your team. Each medicine can need a different change.

BASELINE → TREATMENT → FOLLOW-UP

Checks across the whole treatment course

The columns show the order of care, not equal lengths of time. Confirm who books each check, who reviews it and whom to call if it is missing.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00210 v9b · official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

FOLFOX-4: treatment and checks

Colorectal cancer

Day 1: oxaliplatin and folinic acid, then fluorouracil bolus and a 22-hour infusion. Day 2: further fluorouracil bolus and a second 22-hour infusion; completion around Day 3. A repeating 14-day cycle. For adjuvant FOLFOX-4, NCCP describes 12 cycles; metastatic treatment continues while suitable.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Blood count / kidneys / liver NCCP 00210	Baseline bloods; DPD before fluoropyrimidine	Repeat before each cycle	Earlier tests if unwell, dehydrated or toxicity develops	Results and recovery guide any delay or dose change	Follow unresolved abnormalities
Symptoms and function NCCP 00210	Document existing symptoms and usual activity	Review worst symptoms since the last treatment	Use urgent contacts; do not wait for the next visit	Balance benefit, side effects and priorities	Agree recovery care and ongoing support
Neuropathy / cold sensitivity NCCP 00210	Record existing nerve symptoms	Formal neuropathy evaluation every two cycles; review symptoms sooner if they occur	Report worsening numbness or cold-triggered throat symptoms	Consultant reviews the oxaliplatin component separately	Nerve symptoms can continue after treatment; arrange review

Care / check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Heart checks NCCP 00210	Review cardiac history; electrocardiogram (ECG) when cardiac function is compromised or clinical findings warrant it. No automatic serial echocardiogram schedule is specified for this regimen.	Ask about new chest pain or breathlessness	Chest pain needs urgent assessment	Repeat heart tests if clinically indicated	Follow any heart problem; no fixed echo series
Response / scans NCCP 00210	Confirm cancer setting, disease assessment and intended duration	Review clinical benefit and symptoms	New symptoms may bring review forward	Consultant-directed scans and response assessment	Agree follow-up, completion or another treatment

What each check is for

1 A 14-day reference cycle

Clinic → home → review

FOLFOX-4 may be given after surgery for stage II or III colon cancer, or for colorectal cancer that has spread. Your team explains the aim, alternatives and how benefit will be reviewed.

Ask / act: Confirm the phase, prescription, tests and supportive medicines before starting.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your port and treatment pump

From preparing the line to coming home after disconnection. Your team arranges the appointments and shows you what to do.

1 · Before treatment

Choose and prepare the line

The team explains whether a port or peripherally inserted central catheter (PICC) suits your treatment. You receive instructions for the insertion appointment.

2 · Insertion and recovery

A separate appointment may be needed

For a port, numbing medicine is used and a small device is placed under the skin. Ask about sedation, transport and wound care.

3 · Day 1

First 22-hour infusion

After the day-ward medicines and fluorouracil bolus, trained staff connect the first 22-hour infusion.

4 · Day 2

Second treatment and infusion

Return as arranged for further fluorouracil bolus and a second 22-hour infusion. The team confirms the setting and any other Day 2 medicines.

5 · Around Day 3

Professional disconnection

Staff check completion of the second infusion and care for the line. Do not use a single 46-hour pump timetable.

6 · Between cycles

The access device usually stays

A port needle can be removed while the port stays under the skin. A PICC remains in the arm with ongoing dressing and line care.

Port or PICC: what is the difference?

A port is a small chamber beneath the skin, usually on the chest, attached to a tube ending in a large vein. A special needle goes through the skin into the port when it is used. A PICC enters a vein in the upper arm; a short section remains outside under a dressing. Both can deliver a home infusion. Your team helps choose the device and teaches its care.

Before insertion and during recovery

Tell the team about allergies, blood thinners, infections and any other cancer medicines. Follow the instructions about eating, drinking and transport; do not stop medicines yourself. Port placement uses local anaesthetic, sometimes with sedation or general anaesthetic. The team explains the procedure and risks, including infection, bleeding, clots and insertion complications. Ask how to manage discomfort, protect the wound and resume activity. Report increasing pain, redness, swelling, discharge or fever.

Connection: what the nurse does

The nurse checks the line and treatment, secures the connection and dressing, and starts the pump. Before leaving, you should have the disconnection location and time, contact numbers including out of hours, and the written spill instructions. Ask to be shown what normal pump progress looks like. You do not need to calculate a rate.

Carrying it, sleeping and washing

Use the supplied carrier and the position recommended for your pump. Keep tubing free from pulling and kinks. Ask how to place the pump safely for sleep and how to protect the line and dressing when washing. Do not immerse it or swim. Avoid direct heat and cold. An elastomeric pump has a balloon that shrinks slowly and may have no alarm; an electronic pump has different instructions. Follow the instructions for the device you actually receive.

Disconnection is not port removal

Keep the appointment even if the pump looks empty, unless the team changes the plan. Staff check the pump and line and arrange safe disposal. After flushing and other line care as required, the port needle may be removed and a small dressing applied. The implanted port stays in place until a separate removal procedure is arranged. A PICC remains in place and needs scheduled dressing and flushing care.

Leakage, disconnection or unexpected pump progress

Call the team immediately if the pump leaks, is damaged, finishes unexpectedly early, appears not to run, or the line disconnects. Follow the emergency or spill steps you have been specifically taught, including use of a clamp if instructed. Do not reconnect, change settings or disconnect the pump yourself. Keep other people away from leaked medicine. If medicine touches skin, rinse it off and wash with soap and water, then seek advice.

Pain, swelling or feeling unwell

Contact oncology immediately for pain or swelling around the line, a wet/loose dressing, redness, fever, shivering or suddenly feeling unwell. A painful swollen arm or neck needs urgent advice. Severe chest pain, major breathing difficulty or collapse: call 112 / 999. Do not wait for pump disconnection or the next clinic visit.

The drawings show general steps, not instructions for inserting or accessing a line. Your hospital confirms its own arrangements and the care instructions for your device.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before the first treatment

Confirm the aim, likely course length, blood-test arrangements and a working urgent contact number. DPD testing is required before starting the fluoropyrimidine unless already completed.

STEP 2

Line and pump arrangements

FOLFOX4 has separate fluorouracil infusions over 22 hours on Day 1 and again on Day 2. Confirm both attendance/pump arrangements and the final disconnection; this is not the single 46-hour modified FOLFOX6 schedule.

STEP 3

If treatment is delayed

Ask the unit to confirm the new treatment and blood-test dates. Re-anchor supportive medicines to the treatment actually given; do not take an extra or catch-up anticancer dose yourself.

STEP 4

Getting help between visits

Follow your oncology unit’s urgent advice for fever, infection, severe diarrhoea, chest pain or difficulty breathing. Bring your treatment card and current medicines list.

STEP 5

Paper copies, wallet QR and contacts

The example telephone numbers on the wallet card are placeholders. Your team must replace them with working daytime and out-of-hours numbers before patient use. Keep the card with you; the QR opens the regimen information, not your confidential medical record.

Before the first treatment

Checks reduce avoidable risk.

Blood tests and medicines

Review blood counts, kidney/liver tests, symptoms, allergies and all medicines and supplements. Confirm DPD testing before fluorouracil. Kidney impairment can affect uracil-based DPD testing interpretation.

Preparing access

Arrange Day 2 infusion renewal and final disconnection around Day 3. This is two separate 22-hour infusions; do not use a 46-hour pump timetable.

Visits and treatment sequence

Know which steps happen in the clinic and at home.

Radiotherapy relationship

This NCCP regimen does not contain a routine radiotherapy course. If radiation is recommended separately, ask whether it is before, during or after these medicines.

When treatment changes or ends

Follow-up continues.

Symptoms and monitoring

Persistent numbness after oxaliplatin needs follow-up. Ask which medicines to continue, when to seek help and how response will be assessed.

Next decisions

The team reviews benefit, side effects and your priorities. Supportive and palliative care can help alongside active treatment. Ask for a clear next-step and contact plan.

Symptoms and supportive care

FOLFOX-4 · NCCP 00210

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Day-ward chemotherapy	 **Day 1** intravenous (IV)
First fluorouracil infusion	 **Days 1–2** About 22 hours
Day 2 treatment / second infusion	 **Days 2–3** Further bolus + about 22 hours
Cold-triggered sensations	 **Days 1–5** Often early; timing varies
Persistent nerve symptoms	 **Days 1–14** Can accumulate over cycles
Blood counts / infection risk	 **Days 1–14** Blood tests and symptoms guide review
Energy and bowel symptoms	 **Days 1–14** Variable timing

Your planned visits: Day 1: oxaliplatin and folinic acid, then fluorouracil bolus and a 22-hour infusion. Day 2: further fluorouracil bolus and a second 22-hour infusion; completion around Day 3. A repeating 14-day cycle. For adjuvant FOLFOX-4, NCCP describes 12 cycles; metastatic treatment continues while suitable.

Care continues: Numbness or tingling can persist after oxaliplatin; tell your team if it affects daily tasks. Ask which medicines to continue and when your next review is planned.

No guaranteed safe days: An infection, reaction or other urgent problem can occur at any time. The graph does not predict a personal low point.

Why these bands stay broad: NCCP defines the treatment days. Symptom bands are broad educational guidance; reactions can occur on any day, and cumulative effects may continue between cycles. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00210 v9b · official regimen ↗

There are no guaranteed "safe days". Fever, shivers or suddenly feeling unwell require immediate contact. Serious symptoms override any timeline or diary.

Between-treatment notes

Day 1: Treatment and checks

The team reviews blood tests, symptoms and your prescription before treatment. Know the contact number and the next planned visit.

Day 2–3: Day 2 treatment and completion

A second fluorouracil treatment is planned on Day 2; trained staff renew the infusion. The team confirms the setting and any other Day 2 medicines. Call early for symptoms; do not wait for the next appointment.

Day 4–14: Recovery and the next review

The team uses symptoms, tests and your priorities to decide what happens next. Bring questions and a symptom record if useful; this is optional.

Support around the medicines

1 Nausea prevention and rescue

Take the prescribed prevention medicines at the planned times and keep rescue treatment available. Tell the team if nausea or constipation is not controlled; do not duplicate steroid or anti-sickness medicines.

2 Bowel symptoms

Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.

3 Counts and recovery

White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.

4 Cold and nerve symptoms

Avoid cold exposure as advised after oxaliplatin. Report persistent numbness or difficulty with buttons, walking or everyday tasks before the next dose.

Symptoms in context

Report what changed and how it affects you.

Bowel symptoms

Follow your written rescue plan. Fever, blood, severe pain, reduced urine or inability to drink needs urgent advice.

Skin or nerve changes

Avoid cold drinks and cold exposure as instructed after oxaliplatin; do not suck ice chips during fluorouracil in this regimen. Use prescribed anti-sickness medicines, pace activities and ask for nutritional, emotional or practical support.

Living with treatment

Use the support available.

Pregnancy and fertility

Discuss fertility preservation and reproductive plans before treatment. Ask for the contraception and breastfeeding instructions for every medicine; the whole regimen may require longer precautions than one medicine alone.

The nerve-check schedule

Reporting symptoms and scheduled assessment work together.



Formal review every two cycles

This NCCP regimen specifies neuropathy evaluation every two cycles. Report worsening numbness, tingling or trouble with buttons or walking immediately rather than waiting for that review.

Cold precautions

Oxaliplatin can cause cold-triggered tingling and throat sensations. Avoid cold triggers and do not use ice chips. New breathing difficulty still requires assessment; it must not be assumed to be only cold sensitivity.

Questions worth asking

Do I need to enter dates to understand the cycle?

No. The graph explains a general pattern. Your hospital arranges appointments; the guide and its safety information are available without entering anything.

What happens if a dose is delayed?

The team checks the cause and may delay, reduce, change or stop one medicine. Ask which component changes and when you should next attend; do not restart yourself.

How will I know whether it is helping?

Scans, symptoms and clinical review assess benefit. A rash, side effect or good day alone cannot show whether the cancer is responding.

Why is Day 2 different from other FOLFOX leaflets?

NCCP FOLFOX-4 has fluorouracil on Days 1 and 2, each with a 22-hour infusion. It is not modified FOLFOX-6. The team confirms your Day 2 prescription and visit; do not rely on another FOLFOX schedule.

> **Try explaining the plan back:** Can you explain which visits are planned, whether a pump is involved, and which symptoms mean you should call today? No dates or symptom scores are needed to use this guide.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

FOLFOX-4 · active treatment cycle

Active medicines: Oxaliplatin + Folinic acid + Fluorouracil

Emetogenic risk: Moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally once	Before Day 1 chemotherapy Review QT risk, interactions, constipation and hepatic function.
Dexamethasone	8 mg orally once	Before Day 1 chemotherapy Reconcile all steroid prescriptions; do not duplicate.
Dexamethasone	8 mg orally once daily	Days 2 and 3 after Day 1 chemotherapy Confirm the prescribed delayed prophylaxis and re-anchor it if treatment is delayed.
Metoclopramide	10 mg orally up to three times daily as required	Rescue only; duration to be completed by prescriber Review renal function, neurological contraindications and interactions.

Additional phase-specific support

- Take the prescribed prevention medicines at the planned times and keep rescue treatment available. Tell the team if nausea or constipation is not controlled; do not duplicate steroid or anti-sickness medicines.
- Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.
- White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.
- Avoid cold exposure as advised after oxaliplatin. Report persistent numbness or difficulty with buttons, walking or everyday tasks before the next dose.
- FOLFOX4 has Day 1 and Day 2 fluorouracil/folinic acid. Reconcile antiemetic cover across both days; do not issue a second independent Day 1 steroid course automatically.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Day 2 · FOLFOX4 fluorouracil/folinic acid

Active medicines: Folinic acid + Fluorouracil

Emetogenic risk: Low treatment-day risk; delayed Day 1 moderate-risk coverage still applies

Medicine	Dose / route	When
Dexamethasone	8 mg PO once	Day 2; already part of the Day 1 moderate-risk Days 2–3 plan Reconcile this single dose across both plans; do not issue an additional duplicate dose.
Metoclopramide	10 mg PO up to three times daily as required	Rescue only as prescribed Review persistent nausea, hydration and individual contraindications.

Additional phase-specific support

- FOLFOX4 Day 2 includes folinic acid and fluorouracil bolus followed by a second 22-hour infusion.
- No repeat oxaliplatin is scheduled on Day 2.
- NCIS multi-day guidance: cover the risk on each administered treatment day and delayed risk after treatment, using a reconciled prescription.
- Take the prescribed prevention medicines at the planned times and keep rescue treatment available. Tell the team if nausea or constipation is not controlled; do not duplicate steroid or anti-sickness medicines.
- Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.
- White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.
- Avoid cold exposure as advised after oxaliplatin. Report persistent numbness or difficulty with buttons, walking or everyday tasks before the next dose.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Nerves

Oxaliplatin

Cold-triggered tingling or difficulty with buttons/walking: report.

2 Mouth / swallowing

Fluorouracil

Pain or ulcers affecting eating or drinking need advice.

3 Breathing / reaction

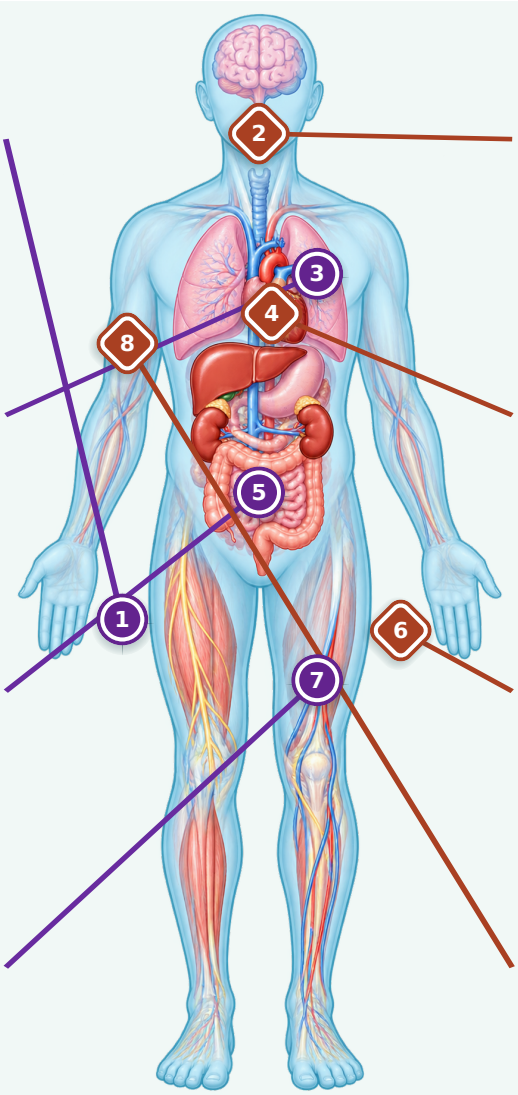
Treatment

New breathing difficulty requires prompt assessment.

4 Chest

Fluorouracil

New chest pain needs urgent assessment.



5 Bowel

Treatment

Diarrhoea, increased stoma output or dehydration: call early.

6 Skin / nails

Fluorouracil

Report troublesome skin changes or painful hands/feet.

7 Blood / infection

Treatment

Fever, shivers or sudden illness: contact oncology immediately.

8 Line / pump

Treatment delivery

Pain, swelling, leakage or a loose connection needs immediate advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

New breathing difficulty, chest pain, collapse, severe diarrhoea or inability to drink require prompt assessment. Call for line pain, swelling, leakage or unexpected pump progress.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

SACTCHECK · CHAPTER 04

Your record and contact card

FOLFOX-4 · National Cancer Control Programme (NCCP) 00210

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Oxaliplatin · Platinum chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Numbness / tingling	□ None	□ Mild change; usual tasks unaffected	□ Limits buttons, writing or walking: call	□ Washing, dressing or walking difficult: urgent review	□ Sudden major weakness / paralysis: emergency help
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999

Folinic acid · Chemotherapy modulator

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999

Fluorouracil · Chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Hand-foot soreness	□ None	□ Redness or tingling without pain	□ Pain limits usual tasks: call	□ Pain limits washing / dressing: urgent review	□ Acute severe illness: emergency help

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Keep your treatment details to hand

Colorectal cancer · NCCP 00210 v9b · Document revised 4 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

FOLFOX-4

Every 14 days · two successive 22-hour fluorouracil infusions · course length confirmed by team.

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Rolling record — follow your agreed treatment plan.

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.33 · NCCP 00210 v9b · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

Sources and further information

FOLFOX-4 · National Cancer Control Programme (NCCP) 00210

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00210 v9b · official regimen**
   https://healthservice.hse.ie/documents/6550/210_v9b_FOLFOX4.pdf
2. **eviQ: implanted port**
   https://www.eviq.org.au/p/507
3. **eviQ: peripherally inserted central catheter (PICC)**
   https://www.eviq.org.au/p/543
4. **eviQ: infusion pump care**
   https://www.eviq.org.au/p/502
5. **HSE: chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
6. **MOSAIC ten-year follow-up — primary report**
   https://pubmed.ncbi.nlm.nih.gov/26527776/
7. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq
8. **NCCP NCIS antiemetics V8, Table 1; apply oral regimen guidance separately**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf